

SURGICAL PATHOLOGY REPORT

UID:0F0F3285-4798-408F-95D3-20AB4F0ED269
TCGA-XF-AAN0-01A-PR

Redacted



Collected
Ordered by

Location

DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY, BILATERAL PELVIC LYMPH NODE DISSECTION AND URINARY DIVERSION (T-POUCH):

- RIGHT DISTAL URETER (A):
FROZEN SECTION DIAGNOSIS:
- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED
FINAL DIAGNOSIS:
- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

- LEFT DISTAL URETER (B):
FROZEN SECTION DIAGNOSIS:
- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED
FINAL DIAGNOSIS:
- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

- PARA-AORTIC PROXIMAL LIMITS LYMPH NODES (C):
FROZEN SECTION DIAGNOSIS:
- BENIGN
FINAL DIAGNOSIS:
- NO MALIGNANCY IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

- APICAL URETHRAL MARGIN (D):
FROZEN SECTION DIAGNOSIS:
- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED
FINAL DIAGNOSIS:
- SCANT MALIGNANT CELLS IN PERIPHERAL SOFT TISSUE OF URETHRA (SEE COMMENT)
- BENIGN URETHRAL MUCOSA

BLADDER AND PROSTATE (E):

BLADDER:

- INVASIVE POORLY DIFFERENTIATED UROTHELIAL CARCINOMA WITH FOCAL SIGNET RING CELL COMPONENT (3.5 X 3.0 X 2.2 CM GROSS DIMENSIONS), GRADE 4/4, GROSSLY AND MICROSCOPICALLY EXTENDING THROUGH ANTERIOR AND RIGHT AND LEFT LATERAL BLADDER WALLS TO INVOLVE PERIVESICAL SOFT TISSUE
- LYMPHOVASCULAR INVASION IDENTIFIED
- UROTHELIAL CARCINOMA IN SITU (FLAT LESION) IDENTIFIED INCLUDING PAGETOID INVOLVEMENT OF TRIGONE
- RANDOM BLADDER MUCOSA EXHIBITING FOCI OF MODERATE UROTHELIAL DYSPLASIA AND REACTIVE ATYPIA
- INKED BLADDER RESECTION MARGINS, FREE OF MALIGNANCY

PROSTATE:

- INVASIVE UROTHELIAL CARCINOMA INVOLVING PERIPROSTATIC SOFT TISSUE OF ENTIRE PROSTATE, WITH INVASION OF PROSTATIC STROMA AND LYMPHOVASCULAR SPACE INVOLVEMENT
- INVASIVE UROTHELIAL CARCINOMA INVOLVING SOFT TISSUE AROUND SEMINAL VESICLES
- UROTHELIAL CARCINOMA INVOLVING INKED RIGHT POSTERIOR PERIPROSTATIC SOFT TISSUE MARGIN
- RARE FOCI OF HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN II)
- NO PROSTATIC ADENOCARCINOMA IDENTIFIED

RIGHT PARACAVAL LYMPH NODES (F):

- METASTATIC CARCINOMA IDENTIFIED IN THREE OF THREE LYMPH NODES EXAMINED (3/3)

ICD-O-3

Carcinoma, urothelial NOS
8120/3

Site: Bladder NOS
C67.9

9/23/26/14

Pw TSS, 0% signet ring.

SURGICAL PATHOLOGY REPORT

Collected:

Ordered by:

Location:

LEFT PARA-AORTIC LYMPH NODES (G):

- METASTATIC CARCINOMA IDENTIFIED IN ONE OF THREE LYMPH NODES EXAMINED (1/3)

RIGHT COMMON ILIAC LYMPH NODES (H):

- METASTATIC CARCINOMA IDENTIFIED IN TWO OF TWO LYMPH NODES EXAMINED (2/2)

LEFT COMMON ILIAC LYMPH NODES (I):

- METASTATIC CARCINOMA IDENTIFIED IN THREE OF SEVEN LYMPH NODES EXAMINED (3/7)

RIGHT LYMPH NODES OF CLOQUET (J):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

RIGHT EXTERNAL ILIAC LYMPH NODES (K):

- METASTATIC CARCINOMA IDENTIFIED IN THREE OF FOUR LYMPH NODES EXAMINED (3/4) [LARGEST POSITIVE LYMPH NODE 2.9 CM]

RIGHT OBTURATOR/HYPOGASTRIC LYMPH NODES (L):

- METASTATIC CARCINOMA IDENTIFIED IN SIX OF NINE LYMPH NODES EXAMINED (5/8)

LEFT EXTERNAL ILIAC LYMPH NODES (M):

- METASTATIC CARCINOMA IDENTIFIED IN TWO OF SEVEN LYMPH NODES EXAMINED (2/7)

LEFT LYMPH NODE OF CLOQUET (N):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

LEFT OBTURATOR/HYPOGASTRIC LYMPH NODES (O):

- METASTATIC CARCINOMA IDENTIFIED IN FOUR OF FOUR LYMPH NODES EXAMINED (4/4)

RIGHT PRESACRAL LYMPH NODES (P):

- METASTATIC CARCINOMA IDENTIFIED IN THREE OF THREE LYMPH NODES EXAMINED (3/3)

PRESACRAL LYMPH NODES (Q):

- METASTATIC CARCINOMA IDENTIFIED IN TWO OF FOUR LYMPH NODES EXAMINED (2/4)

LEFT PRESACRAL LYMPH NODES (R):

- METASTATIC CARCINOMA IDENTIFIED IN TWO OF FOUR LYMPH NODES EXAMINED (2/4)

RIGHT PROXIMAL URETER (S):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT PROXIMAL URETER (T):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LYMPH NODE SUMMARY: METASTATIC CARCINOMA IDENTIFIED IN TOTAL OF 30 OUT
53 LYMPH NODES EXAMINED (30/53)

PATHOLOGIC TNM STAGE: pT4aN2MX

Electronically signed by
Verified:

SURGICAL PATHOLOGY REPORT

Collected: [REDACTED]

Ordered by: [REDACTED]

Location: [REDACTED]

COMMENT:

Dr. [REDACTED] was notified of the discrepancy in the frozen section diagnosis and final diagnosis for the apical urethral margin,

SPECIMEN SOURCE:

A: "Right distal ureter"
 B: "Left distal ureter"
 C: "Para aortic proximal limits lymph nodes - FS"
 D: "Apical urethral margin"
 E: "Bladder + prostate"
 F: "Right para caval lymph nodes"
 G: "Left para aortic lymph nodes"
 H: "Right common iliac lymph nodes"
 I: "Left common iliac lymph nodes"
 J: "Right lymph nodes of cloquet"
 K: "Right external iliac lymph nodes"
 L: "Right obturator/hypogastric lymph nodes"
 M: "Left external iliac lymph nodes"
 N: "Left lymph node of cloquet"
 O: "Left obturator/hypogastric lymph nodes"
 P: "Right pre sciatic lymph nodes"
 Q: "Pre sacral lymph nodes"
 R: "Left pre sciatic lymph nodes"
 S: "Right proximal ureter"
 T: "Left proximal ureter"

CLINICAL INFORMATION:

Pre-Op Dx: Bladder cancer, TCC

GROSS EXAMINATION:

A: The specimen is received fresh from the O.R. and labeled "Right distal ureter". It consists of a segment of ureter measuring 0.4 cm in length by 0.2 cm in diameter. The specimen is submitted entirely for frozen section in cassette AFS.

B: The specimen is received fresh from the O.R. and labeled "Left distal ureter". It consists of a segment of ureter measuring 0.4 cm in length by 0.2 cm in diameter. The specimen is submitted entirely for frozen section in cassette BFS.

C: The specimen is received fresh from the O.R. and labeled "Para aortic proximal limits lymph nodes - FS". It consists of a segment of soft tan-yellow adipose tissue measuring 2 x 1.1 x 0.8 cm. Multiple lymph nodes are identified and submitted entirely for frozen section in cassette CFS.

D: The specimen is received fresh from the O.R. and labeled "Apical urethral margin". It consists of two fragments of soft tan-pink tissue measuring in aggregate 1.4 x 0.4 x 0.4 cm. The specimen is submitted entirely for frozen section in cassette DFS.

E: The specimen is received fresh from the O.R. and labeled "Bladder + prostate". It consists of a cystoprostatectomy specimen measuring overall 14 x 10.5 x 4.8 cm. The peritoneum measures 15 x 11 x 0.1 cm and is smooth, glistening and freely mobile. The non-peritonealized surface is inked black with red ink superimposed over the right side of the prostate. The specimen is opened anteriorly along the prostatic urethra. The bladder measures 6 x 4 x 3.5 cm with a wall thickness of 0.8 cm. The bilateral ureterovesical junctions are identified and are patent. The ureters are then opened. The right attached ureter measures 5 cm in length by 0.5 cm in circumference and the left attached ureter measures 4 cm in length by 0.5 cm in circumference. Both the right and left ureters are grossly unremarkable. The

SURGICAL PATHOLOGY REPORT

Collected:

Ordered by:

anterior wall of the bladder is replaced by a tan-red, firm and ulcerated mass that extends from the anterior wall to both the right and left lateral aspects of the bladder while sparing the ureterovesical junctions and trigone area. This mass measures 3.5 x 3 x 2.2 cm and appears to grossly involve the full thickness of the bladder wall with gross extension into the perivesical fat anteriorly and on both the right and left aspects of the bladder. The wall thickness of the involved bladder measures 2.2 cm. The remaining bladder mucosa is tan-pink and slightly edematous. The prostate measures 4 x 3.5 x 3.5 cm. The prostatic urethra measures 3 cm in length by 2 cm in circumference and is grossly unremarkable. The verumontanum measures 1 x 0.3 x 0.3 cm. The right seminal vesicle measures 3 x 1.5 x 1 cm, and the left seminal vesicle measures 3 x 1.5 x 1 cm. Both the seminal vesicles are grossly unremarkable. Palpating the perivesical fat shows no additional nodules. Representative sections are submitted in 27 cassettes.

F: The specimen is received in formalin and labeled "Right para caval lymph nodes". It consists of a segment of soft tan-yellow fibroadipose tissue measuring 4 x 2 x 1 cm. Two lymph nodes are identified, the largest measuring 2 cm in greatest dimension. The specimen is submitted entirely in three cassettes.

G: The specimen is received in formalin and labeled "Left para aortic lymph nodes". It consists of a single segment of soft tan-yellow fibroadipose tissue measuring 3.5 x 1 x 1 cm. One lymph node is identified measuring 1 cm in greatest dimension. The specimen is submitted entirely in one cassette.

H: The specimen is received in formalin and labeled "Right common iliac lymph nodes". It consists of two segments of soft tan-yellow fibroadipose tissue measuring in aggregate 6 x 3 x 1 cm. Multiple lymph nodes are identified, the largest measuring 3.8 cm in greatest dimension. The specimen is submitted entirely in six cassettes.

I: The specimen is received in formalin and labeled "Left common iliac lymph nodes". It consists of multiple segments of soft tan-yellow fibroadipose tissue measuring 4 x 1.5 x 2 cm in aggregate. Multiple lymph nodes are identified, the largest measuring 1.8 cm in greatest dimension. The specimen is submitted entirely in three cassettes.

J: The specimen is received in formalin and labeled "Right lymph nodes of cloquet". It consists of a single segment of soft tan-yellow fibroadipose tissue measuring 2 x 1.1 x 0.7 cm. One lymph node is identified measuring 1.5 cm in greatest dimension. The specimen is bisected and submitted entirely in one cassette.

K: The specimen is received in formalin and labeled "Right external iliac lymph nodes". It consists of multiple segments of soft tan-yellow fibroadipose tissue measuring in aggregate 5 x 3.5 x 1 cm. Multiple lymph nodes are identified, the largest measuring 3 cm in greatest dimension. The specimen is submitted entirely in six cassettes.

L: The specimen is received in formalin and labeled "Right obturator/hypogastric lymph nodes". It consists of multiple segments of soft tan-yellow fibroadipose tissue measuring in aggregate 6.5 x 4 x 2 cm. Multiple lymph nodes are identified, the largest measuring 4 cm in greatest dimension. The specimen is submitted entirely in eight cassettes.

M: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of multiple pieces of soft tan-yellow fibroadipose tissue measuring in aggregate 4 x 3 x 3 cm. Multiple lymph nodes are identified, the largest measuring 2 cm in greatest dimension. The specimen is submitted entirely in seven cassettes.

SURGICAL PATHOLOGY REPORT

Collected
Ordered by

Location

N: The specimen is received in formalin and labeled "Left lymph node of cloquet". It consists of a single segment of tan-yellow fibroadipose tissue measuring 1.5 x 1 x 0.8 cm. One lymph node is identified measuring 1.3 cm in greatest dimension. The specimen is submitted entirely in one cassette.

O: The specimen is received in formalin and labeled "left obturator/hypogastric lymph nodes". It consists of multiple segments of soft tan-yellow fibroadipose tissue measuring in aggregate 6 x 3 x 1 cm. Multiple lymph nodes are identified, the largest measuring 4.6 cm in greatest dimension. The specimen is submitted entirely in six cassettes.

P: The specimen is received in formalin and labeled "Right pre sciatic lymph nodes". It consists of multiple fragments of soft tan-yellow fibroadipose tissue measuring in aggregate 2 x 1 x 0.6 cm. Two lymph nodes are identified, the largest measuring 0.5 cm in greatest dimension. The specimen is submitted entirely in one cassette.

Q: The specimen is received in formalin and labeled "Pre sacral lymph nodes". It consists of a single piece of soft tan-pink fibroadipose tissue measuring 4.8 x 2.5 x 0.8 cm. No discrete masses are identified. The specimen is submitted entirely in three cassettes.

R: The specimen is received in formalin and labeled "Left pre sciatic lymph nodes". It consists of multiple fragments of soft tan-yellow fibroadipose tissue measuring in aggregate 2.5 x 1.5 x 1 cm. Multiple lymph nodes are identified, the largest measuring 0.4 cm in greatest dimension. The specimen is submitted entirely in one cassette.

S: The specimen is received in formalin and labeled "Right proximal ureter". It consists of a segment of ureter with surrounding adipose tissue measuring overall 1.5 x 1 x 0.4 cm. The ureter measures 0.8 cm in length by 0.2 cm in average diameter. Two metallic clips are present equidistant from both ends. The specimen is bisected and submitted entirely in one cassette.

T: The specimen is received in formalin and labeled "Left proximal ureter". It consists of a segment of ureter with attached adipose tissue measuring overall 1.5 x 1 x 0.6 cm. The ureter measures 1 cm in length by 0.2 cm in average diameter. Two metallic clips are present along one end of the ureter. The end opposite the ureter is inked black and the specimen is trisected and submitted entirely in one cassette.

SECTIONS:

AFS:	frozen section, right distal ureter
BFS:	frozen section, left distal ureter
CFS:	frozen section, para-aortic proximal limits lymph nodes
DPS:	frozen section, apical ureteral margin
E1-3:	bladder and prostate; right anterior and lateral wall of bladder
E4-6:	left anterior and lateral wall of bladder
E7:	dome
E8:	posterior wall of bladder
E9:	right ureterovesical junction
E10:	left ureterovesical junction
E11:	trigone
E12:	urethral junction
E13:	right perivesical fat for possible lymph nodes
E14:	left perivesical fat for possible lymph nodes
E15:	prostate, right apex anterior
E16:	prostate, right apex posterior
E17:	prostate, left apex anterior and posterior
E18:	prostate, right mid anterior

SURGICAL PATHOLOGY REPORT

Collected:

Ordered by:

Location:

E19: prostate, right mid posterior
 E20: prostate, left mid anterior
 E21: prostate, left mid posterior
 E22: prostate, right base anterior
 E23: prostate, right base posterior
 E24: prostate, left base anterior
 E25: prostate, left base posterior
 E26: right seminal vesicle junction with prostate
 E27: left seminal vesicle junction with prostate
 F1,2: right paracaval lymph nodes; largest lymph node, bisected
 F3: one lymph node, bisected
 G: left para-aortic lymph nodes - all embedded
 H1-4: right common iliac lymph nodes; one large lymph node quadrisectioned
 H5: one lymph node, bisected
 H6: remaining tissue
 I1: left common iliac lymph nodes; multiple lymph nodes
 I2,3: remaining tissue
 J: right lymph nodes of Cloquet; one lymph node, bisected
 K1,2: right external iliac lymph nodes; largest lymph node, bisected
 K3: one lymph node, bisected
 K4: one lymph node, bisected
 K5,6: remaining tissue
 L1-4: right obturator/hypogastric lymph nodes; one large lymph node quadrisectioned
 L5: one lymph node, bisected
 L6: one lymph node, bisected
 L7: three lymph nodes
 L8: remaining tissue
 M1: left external iliac lymph nodes; largest lymph node, bisected
 M2: one lymph node, bisected
 M3: one lymph node, bisected
 M4: multiple lymph nodes
 M5-7: remaining tissue
 N: left lymph node of Cloquet; one lymph node, bisected
 O1-4: left obturator/hypogastric lymph node; one large lymph node quadrisectioned
 O5: one lymph node, bisected
 O6: remaining tissue
 P: right presciatic lymph nodes - all embedded
 Q1-3: presacral lymph nodes - all embedded
 R: left presciatic lymph nodes - all embedded
 S: right proximal ureter - all embedded
 T: left proximal ureter - all embedded

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter:
 - no high grade atypia or tumor identified

 BFS: Left distal ureter:
 - no high grade atypia or tumor identified

 CFS: Para-aortic proximal limits lymph nodes:
 - benign

 DFS: Apical ureteral margin:
 - no high grade atypia or tumor identified

Collected
Ordered by

Location

MICROSCOPIC EXAMINATION:

A-C: See final microscopic-diagnoses.

D: Permanent sections of the apical urethral margin show single and small clusters of malignant cells in the periurethral soft tissue, particularly around and within muscle bundles. A review of the original frozen sections shows rare atypical cells, that when viewed in context with the main tumor in the bladder and the permanent sections of the urethral margin are interpreted as representing carcinoma. The urethral mucosa itself and the immediate underlying support tissue shows no evidence of urothelial dysplasia or malignancy.

E: Sections of the bladder show an invasive poorly differentiated urothelial carcinoma composed of cells with enlarged hyperchromatic nuclei with coarse chromatin and prominent central macronucleoli (E1-6). Some of the tumor cells display signet ring cell features. The tumor invades through the muscularis propria into the perivesical fat and involves the entire anterior bladder wall as well as the right and left lateral bladder wall regions. Urothelial carcinoma in situ (flat lesion) is identified in association with the invasive carcinoma (E4,5) as well as in the trigone region where it is represented by pagetoid spread. Other random sections of bladder mucosa exhibit foci of moderate urothelial dysplasia and areas of reactive atypia. The inked bladder resection margins are free of malignancy.

Sections of the prostate show extensive urothelial carcinoma involving periprostatic soft tissue (E17-23,25) with invasion of prostatic stroma from "the outside in" (E18-21). Lymphovascular invasion is seen (E21,25). Tumor extends to the right posterior inked margin of the prostate (E19,23). Tumor involves tissue around the seminal vesicles without involving the seminal vesicles themselves (E26,27). The uninvolved prostate shows cystic atrophy and rare foci of high grade prostatic intraepithelial neoplasia (PIN II). No prostatic adenocarcinoma is seen.

F-T: See final microscopic-diagnosis

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle) <u>QUALIFIED</u> / DISQUALIFIED		
Reviewer Initials <u>W</u> Date Reviewed: <u>3/12/14</u>		